STROKED–OUT SASQUATCH · STROKE RECOVERY ACADEMY



STUCK

The Sasquatch Formula for Breaking a Recovery Plateau

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BEFORE YOU START

Read This First

Four things before you turn the page.

1

Safety first

Nothing in this book replaces your therapist, doctor, or care team. If a drill increases pain, dizziness, or new weakness, stop and call them. You are the scientist here — not the guinea pig for reckless risk.

2

How to use this

Read it once straight through. Then go back and actually fill in the five worksheets by hand — typing them in your head doesn't count. This is a tool you use, not an article you read.

3

What to expect

This won't fix your plateau in one sitting. It will give you a formula and a first move. The breakthrough comes from running T.L.E.C.A. for real, this week, on your actual drill.

4

How often to run it

Every time progress stalls for more than two weeks. Not once. Not as a one-time fix. T.L.E.C.A. is a loop you return to for the rest of your recovery — plateaus don't stop coming, and neither should you.



WELCOME

You Didn't Stall. The Approach Did.



If you're reading this, you already know the feeling this book is named after. Progress was happening — slow, brutal, but real — and then one day it just… stopped. The toe that was starting to twitch stopped twitching. The word that was starting to come back stopped coming. The therapist ran out of new things to try. Everyone around you started saying some version of "this is just where you're at now."

They're wrong. Plateaus aren't the ceiling. They're the signal that the approach you've been running needs to change — not that your recovery is finished.

I hit this wall more times than I can count. What got me through it wasn't a new device or a new doctor. It was a five-step formula I built out of trial, error, and refusing to accept "that's just how it is." I call it the Sasquatch Formula: **T.L.E.C.A.** — Take, Learn, Experiment, Combine, Apply.

This isn't a diagnosis or a replacement for your care team. It's the exact framework I use every time progress stalls, broken down so you can run it on your own plateau — today.

"Plateaus aren't the ceiling. They're the signal."

CHAPTER ONE

The Plateau, Named

Here's what nobody tells you early on: recovery isn't a straight line upward. It's a staircase — flat stretches, then a step, then flat again. The flat stretches are where most people quit. Not because they've actually stopped improving, but because the kind of improvement that's still available to them requires a different move than the one that got them this far.

Motivation got you started. It will not get you through this part. Motivation is a honeymoon phase — the burst of energy in the early weeks when everything is new and every small win feels huge. It fades. What carries you through a plateau is discipline: showing up on the day you feel nothing, see nothing, and want to quit everything.

The plateau is not proof it's over. It's proof the current approach has extracted all the gains it can. A drill that got you your first twitch won't necessarily get you your first grip. A therapy that helped you form words might not be the thing that gets you speaking in full sentences. The work that's required now is different work — and the Sasquatch Formula is how you find it.



SELF-CHECK

How to Know It's a Real Plateau

You're in a real plateau — not just a rough week — if most of these are true:

- [ ] You've done the same drill, the same way, for two or more weeks straight.
- [ ] You've logged no measurable change — no new degree of movement, no new rep, no new word.
- [ ] You've ruled out the five things on the Plateau Audit (fatigue, autopilot dosing, untreated frustration, nutrition, a split goal).
- [ ] Your therapist or doctor confirms nothing has changed clinically that would explain a slowdown.
- [ ] You still feel some pull to keep going — you're frustrated, not finished.

If that's you: good. That means you're not broken — you're exactly at the point where T.L.E.C.A. does its work. Turn to the formula.



MINDSET

Plateau Psychology

Why it feels like failure. Your brain measures progress the way it measures everything else — by what it can see today, compared to yesterday. When two weeks pass with nothing visibly different, the nervous system reads that as "this isn't working" and starts building a case for quitting. That reaction is normal. It is also wrong.

Why it isn't. Neuroplasticity doesn't move in a straight line. Real rewiring happens under the surface — in the reps that don't show a result yet — long before it shows up as a visible gain. A flat line on the outside can be a huge amount of construction on the inside. The output lags the input. That lag is where most people quit, one week before the line would have moved.

How to stay emotionally stable. Measure inputs, not just outputs. Did you show up today? Did you do the rep? That's the number that's actually in your control, and it's the number that predicts whether Day 66 looks different from Day 1.

How to avoid self-blame. A plateau is a data point about the method, not a verdict about your effort or your worth. "This drill has stopped producing gains" is a fact you can act on. "I'm failing at my own recovery" is a story that will stop you from acting at all. Keep the first one. Throw out the second.

How to avoid quitting. Don't decide whether to quit on your worst day. Make the decision on a good day, write it down, and let that decision — not today's mood — run the next 66 days.

MY STORY

Wheelchair to Walmart

I went from a wheelchair to walking out of a Walmart on my own two feet. Not in one leap — in a chain of plateaus, each one broken the same way.

It started with toe taps. Just lifting my toes, over and over, from a wheelchair I couldn't get out of on my own. Then marches — lifting my knees while seated. Then sit-to-stands — just standing up and sitting back down, again and again, until standing stopped being an event.

Every one of those stages plateaued. The toe taps stopped feeling like progress. The marches stopped feeling like progress. Each time, the plan wasn't to do more of the same thing harder — it was to change what I was asking my body to do next. Once sit-to-stands were solid, I didn't keep doing sit-to-stands forever. I started walking — from one chair to another, a few feet at a time, using the next chair as the goal.

Then chair to chair became chair to room. Then room to car. Then car to farther than I thought I could go. Each stage looked like a wall right up until it didn't — and the only way through was the same move every time: take the current stage as far as it would go, then change one thing about what I was asking of myself next.

Eventually that chain of "one more chair, one more room" ended with me walking around a Walmart, on my own, with no wheelchair anywhere in sight.

Mapped to the formula: I **Took** the starting point I had — a wheelchair and toe taps, nothing more. I **Learned** that each stage was only meant to last until it stopped producing gains, not forever. I **Experimented** by changing the target the moment a stage plateaued — marches after toe taps, sit-to-stands after marches, chair-to-chair after sit-to-stands. I **Combined** each new stage with the ground I'd already gained, so chair-to-chair became chair-to-room became room-to-car. And I **Applied** the whole chain daily, without skipping stages, until walking was just what I did.

None of those individual stages looks impressive on its own. A toe tap. A march. A sit-to-stand. Strung together, run past the point each one plateaued, they added up to walking out of a Walmart under my own power. That's the formula. It doesn't skip steps. It just refuses to stop at any one of them.

What Actually Made It Work

Three things made that chain of stages actually work — the raw material every stage was built from.

1. **Weight-bearing**
   Every stage, from sit-to-stands to chair-to-chair, asked my body to bear real weight through the affected side. Weight-bearing is what teaches the nervous system a limb is usable again — it's not optional extra credit.

2. **Stretching**
   Range of motion had to be maintained the whole way, not just built. A joint that's stiff can't take the next stage's demand, no matter how much strength shows up underneath it.

3. **Repetition**
   None of these stages happened once. Toe taps were hundreds of reps before they meant anything. Sit-to-stands were dozens a day for weeks before a single step from chair to chair was possible. Repetition is the tax you pay for the next stage.

And underneath all three: work whatever the stroke will allow, and progress with small, achievable goals. Not "walk again" as Day 1's goal — toe taps as Day 1's goal. Don't be afraid to fail. A sit-to-stand that ends in a stumble, a step that doesn't land clean — those aren't setbacks to the plan. They're the plan. Stack the failures into successes, one small goal at a time, and the chair-to-chair eventually becomes a Walmart with no wheelchair in sight.



CHAPTER TWO

The Sasquatch Formula — T.L.E.C.A.

My recovery wasn't a straight line. It was chaos — trial and error, throwing everything at the wall to see what stuck. T.L.E.C.A. is the battle-tested, five-step system I distilled from that mess. It's a repeatable way to find breakthroughs in exactly the place where most people find a wall.

Most people stop at the first letter. The Sasquatch lives in the third and fourth.

T

Take

Action: Take what the system gives you — the initial therapy, the brace, the standard drill, the conventional advice.
Mindset: Gratitude, but suspicion. Use it as a foundation. Never accept it as the final word. It's your starting point, not your finish line.

L

Learn

Action: Study the science behind the problem. Why does the brain do that? What is neuroplasticity actually doing here?
Mindset: Become the expert in your own brain. When you understand the why, you can engineer the how better than anyone else — ask your therapists the hard questions.

E

Experiment

Action: Use what you learned to modify the standard advice. If the drill is 3 sets of 10, try 1 set of 100. If you're sitting, try standing.
Mindset: Refuse to be a follower. Treat your recovery like a high-stakes science project. Document what works, what fails, and what makes you swear the loudest.

C

Combine

Action: Stack the techniques that worked. Combine weight-bearing with toe taps. Combine mirror therapy with speech drills.
Mindset: Synergy. 1 + 1 must equal 3. Two proven techniques used together produce results greater than either alone.

A

Apply

Action: Integrate the breakthrough into your daily life. Make the new combination the new non-negotiable routine.
Mindset: You aren't applying a cure. You're applying a new operating system. The work never stops — it just changes shape.

T Take

What's the standard advice, drill, or protocol you're currently running?

Who gave you this advice, and when did you last revisit it?

Be honest: are you doing this because it's working, or because it's familiar?



L Learn

What do you actually understand about why this drill or approach works?

What question have you been afraid to ask your therapist or doctor?

Write it down here, then ask it this week. No exceptions.



E Experiment

Name one way you could push this drill further: more reps, more resistance, less rest, a new angle.

What's the smallest safe version of that experiment you could try in the next 24 hours?

What did you notice the last time you tried something 'wrong' on purpose?



C Combine

What's one other drill, tool, or habit you could stack with this one?

What have you already proven works, on its own, that you haven't combined with anything yet?

Remember: 1 + 1 must equal 3. What's the multiplier here?



A Apply

What does doing this look like today — not someday, today?

What time of day will this happen, and what will remind you?

Sign and date this page. This is your contract with yourself.



DEEP DIVE · COMBINE

Combine Examples

Combine is the step people skip — it's the least obvious and the most powerful. Here's what it looks like in practice.

5 EXAMPLE COMBINATIONS

Weight-bearing + toe taps — Stack a static hold with a dynamic drill so the same session trains stability and movement together.

Mirror therapy + speech drills — Run visual feedback and verbal output in the same block — both lean on mirror-neuron activity.

Stim device + compound movement — Use e-stim or a Bioness-style device while walking, not while sitting still — the compound movement recruits more of the pathway.

Cold exposure + grip drill — A brief cold hand soak right before a grip drill can temporarily sharpen sensory feedback for some survivors — test it, don't assume it.

Music + rhythm-based movement — Pairing a steady beat with a repetitive drill can help entrain timing for gait or arm-swing work.

3 "WRONG" COMBINATIONS

Two max-effort drills back to back with no rest — Compounds fatigue instead of compounding gains — you'll just be measuring exhaustion.

Combining techniques your team hasn't cleared — Stacking an unapproved device or medication interaction with a physical drill is a safety risk, not an experiment.

Changing three variables at once — If it works, you won't know why. If it fails, you won't know what to fix.

3 "SAFE" COMBINATIONS

Any two drills you've already proven work alone — If both are individually safe and approved, stacking them is low-risk.

Adding a support element (music, a call, a tracker) to an existing drill — This changes the environment, not the physical risk profile.

Slight dose changes within your therapist's guidance — Small, cleared adjustments to reps or resistance.

3 ADVANCED COMBINATIONS

Tech + compound movement + real-time tracking — Stacking a stim device, a functional movement, and a logged metric in the same session — for survivors well past the basics.

Cross-domain stacking — Combining a physical drill with a cognitive load (counting backward, naming categories) to train under realistic conditions.

Environment variation stacking — Running your best combination in a harder environment — outside instead of the gym, standing instead of seated — once it's solid in the easy version.

QUICK REFERENCE

The Sasquatch Plateau Breakthrough Cheat Sheet

T — Take
Use the standard advice as your floor, not your ceiling.

L — Learn
Understand the why behind the drill — then ask harder questions.

E — Experiment
Change one variable. Document what happens.

C — Combine
Stack two proven techniques. 1 + 1 must equal 3.

A — Apply
Make the winning combination your new non-negotiable.

THE PLATEAU AUDIT

- Fatigue debt
- Same drill, same dose
- Untreated frustration
- Nutrition and hydration
- A goal that's actually two goals

DO THIS IN THE NEXT 24 HOURS

Pick one drill you've run the same way for 2+ weeks. Change ONE variable today.

REMEMBER

Never zero. One small meaningful action still counts.

CHAPTER THREE

The Plateau Audit

Before you run the formula, run this audit. Half of all "plateaus" turn out to be one of these five things in disguise — check each one honestly before assuming you've hit a real wall.

- [ ] **Fatigue debt**
  Are you sleeping enough to actually consolidate the gains you're training for? Neuroplasticity happens during rest, not just during reps.

- [ ] **Same drill, same dose**
  Have you changed anything about this exercise in the last two weeks — reps, resistance, angle — or is it running on autopilot?

- [ ] **Untreated frustration**
  Is fear of failing quietly making you do the drill smaller and safer than you need to, without you noticing?

- [ ] **Nutrition and hydration**
  Are you fueling the rebuild, or running on coffee and whatever's easiest?

- [ ] **A goal that's actually two goals**
  Are you trying to improve grip AND wrist rotation AND finger extension all in the same drill? Split it. One target at a time.



TOOL

How to Document Experiments

T.L.E.C.A. only works if Experiment is repeatable. Use this simple log every time you change a variable — six fields, thirty seconds.

DATE	DRILL	VARIABLE CHANGED	RESULT	FEELING (1–10)	NEXT STEP

DAILY TOOL

The Daily Apply Checklist

Apply only works if it's daily. Five boxes, every day you run the formula.

- [ ] Did I apply the new combination?
- [ ] Did I track the result?
- [ ] Did I adjust the dose based on yesterday?
- [ ] Did I notice a flicker, however small?
- [ ] Did I do one Never Zero action, even on the hard day?



Never zero. One small meaningful action still counts.

DO THIS NOW

Do This in the Next 24 Hours

Reading the formula is not the same as running it. Here is the one action that starts the loop today.

1. **Pick the drill**
   Choose one drill or exercise you've done the same way for two or more weeks.

2. **Change one variable**
   Today, change exactly one thing about it — the angle, the resistance, the speed, the position, or the number of reps. Only one. Not everything at once.

3. **Do it once**
   Run the changed version one time. That's the whole task. Not a program. One rep, one set, one attempt.

4. **Write down what happened**
   Note what you noticed — a new sensation, a small win, nothing at all. All three are useful data.

This is safe, it's measurable, and it takes less than five minutes. It's also the exact first move of the Experiment step. You've already started.



FOR HARD DAYS

The Hard Day Protocol

Some days the plateau doesn't feel like a plateau. It feels like collapse. This page is for those days. Don't run the whole formula today. Just run this.

1. **One micro-action**
   Do the smallest version of your drill that still counts — one rep, thirty seconds, one word. Not zero. Never zero.

2. **One reflection prompt**
   Write one sentence: "Today was hard because ___." That's it. Naming it takes some of its power away.

3. **One grounding line**
   Say out loud: "This is a hard day, not a hard life. I am still rebuilding."

4. **One Never Zero reminder**
   You don't have to feel motivated today. You just have to do the one small thing. The discipline carries you when motivation doesn't show up.

5. **One call-someone option**
   If today feels like more than a hard day — if it feels hopeless, scary, or unsafe — that's not a day to push through alone. Call your care team, a person on your support list, or a crisis line. Reaching out is not giving up. It's the move.

REFRAME

What Progress Actually Looks Like

Most people quit a working plan because they're measuring the wrong kind of progress. Here's what real progress actually looks like — most of it doesn't look like anything at all, at first.

1. **Flickers**
   A twitch, a flash of sensation, a half-formed word. It doesn't look like function yet. It's the first sign the pathway is alive.

2. **Micro-wins**
   One more degree. One more second. One less cue needed. Small enough to dismiss, big enough to log.

3. **Non-linear gains**
   Weeks of nothing, then a jump. This is the normal shape of neuroplastic change, not a sign something's wrong.

4. **Delayed gains**
   The work you're doing today may not show up for months. The gain you see today may be from work you did months ago.

5. **Invisible gains**
   Rewiring that hasn't reached the surface yet. No visible output doesn't mean no input is landing.

6. **Emotional gains**
   Less fear of a movement. More willingness to try. A better day after a string of hard ones. That's recovery too.

7. **Consistency gains**
   Showing up on Day 40 when Day 1 felt impossible. The habit itself is a form of progress, independent of the number.

SAFETY

When to Stop Experimenting

Experimentation needs boundaries. Refusing to be a follower doesn't mean refusing to be safe. Stop and reassess when:

1. **Pain changes character**
   Sharp, new, or worsening pain is not "pushing through" — it's a signal to stop that drill and check with your team.

2. **You need real rest**
   If sleep, fatigue, or stress are compounding, the answer is recovery time, not a harder variable.

3. **Your team hasn't weighed in**
   Any experiment involving a device, medication interaction, or a technique outside your current plan gets cleared by a therapist first.

4. **You're pushing from fear, not curiosity**
   If you're forcing an experiment to prove something to yourself rather than genuinely testing a variable, that's a sign to pause.

5. **The drill stops being safe to do alone**
   Balance, cognitive, or strength demands that exceed what's safe unsupervised mean it's time to switch drills or get a spotter.



WHAT'S NEXT

What to Do After a Breakthrough

A breakthrough isn't the finish line — it's a new baseline that needs to be locked in, or it can slide back.

1. **Stabilize the gain**
   Keep running the combination that produced the breakthrough for at least another 66 days before changing it again. Don't abandon a working formula out of boredom.

2. **Integrate it into daily life**
   Move the new capability out of the drill and into a real task — use the regained grip to hold a cup, not just a therapy ball.

3. **Guard against regression**
   Keep logging. A quiet slide backward is easier to catch in a tracker than in memory.

4. **Build on it**
   Once the gain is stable, that new baseline becomes your next "Take" — the starting point for the next lap through T.L.E.C.A.



BRING THIS WITH YOU

The Therapist Conversation Guide

Most survivors don't know what to ask. Bring this list to your next appointments.

5 Questions for Your Therapist

1. What's the actual mechanism behind this drill — what is it trying to rewire?
2. What would a safe way to increase intensity or novelty on this drill look like?
3. How would we know if this drill has stopped producing gains for me specifically?
4. Is there a version of this drill for a harder environment or a compound movement?
5. What's the earliest sign of a plateau you'd want me to flag to you?

3 Questions for Your Doctor

1. Is there a medical reason (medication, fatigue, an unrelated condition) that could explain a sudden plateau?
2. Are there any restrictions I should know about before I experiment with intensity, angle, or resistance?
3. What symptoms would mean I should stop an experiment immediately and call you?

3 Questions for Your OT

1. What's one everyday task we could use to combine my drill with real-world practice?
2. Is there adaptive equipment that could let me combine two techniques safely?
3. How do we know when a functional gain is ready to be tested outside the clinic?

3 Questions for Your Speech Therapist

1. What's the mechanism behind why this drill helps — what am I actually rebuilding?
2. Could we pair this drill with something else I'm working on, like mirror therapy?
3. What would the next level of difficulty look like once this plateaus?

FOR CAREGIVERS

The Caregiver Version of T.L.E.C.A.

Caregivers run this formula too — from the outside. Here's the caregiver version of each step.

Take

Support how caregivers can help: Learn the current protocol alongside them so you understand what "Take" looks like for their specific plan.

✓ Say: "Walk me through what your therapist gave you — I want to understand it."
✗ Don't say: "Are you sure you're doing that right?"

Learn

Support how caregivers can help: Ask their care team the questions they're too exhausted to ask, and bring back the answers.

✓ Say: "I looked into this — want to hear what I found?"
✗ Don't say: "I read online that you should be doing X instead."

Experiment

Support how caregivers can help: Help track and observe, without pushing for a specific result.

✓ Say: "I noticed you held that a beat longer today — want me to log it?"
✗ Don't say: "Why isn't this working yet?"

Combine

Support how caregivers can help: Help set up combined drills logistically — timers, equipment, a quiet space — without directing the session.

✓ Say: "What do you need from me to make that combination easier to do?"
✗ Don't say: "Just do both at once, it's not that hard."

Apply

Support how caregivers can help: Protect the daily routine — shield the non-negotiable time from other obligations.

✓ Say: "That's your time. I've got everything else covered for those twenty minutes."
✗ Don't say: "Can't you skip it just today?"

How to help without pushing, and how to track progress: keep your own simple log — date, what you observed, how they seemed emotionally — separate from theirs. It gives you language for check-ins with their care team, and it gives them a caregiver who's engaged without looming over every rep.

THE BRAND BEHIND THE FORMULA

The Sasquatch Recovery Identity

Never Zero. On the days you can't do the full drill, do the smallest version that still counts. A streak is just one day, repeated.

One meaningful action. Recovery isn't won in a single heroic session. It's won in the accumulation of small, deliberate actions, repeated past the point where they stop feeling significant.

The Sasquatch mindset. Take what the system gives you, but never mistake it for the ceiling. Stay suspicious of "that's just how it is." Treat your own recovery like the highest-stakes project you'll ever run — because it is.

The rebuild identity. You are not waiting to heal. You are actively rebuilding, one experiment at a time. That's a different posture than "patient" — it's Architect, Builder, Scientist.

The plateau philosophy. Plateaus aren't proof it's over. They're proof the current method has extracted all the gains it can, and it's time for the next formula. Every flat stretch in this book's story ended in a jump. Yours will too.

WHERE TO GO NEXT

Next Steps

This formula is one tool. Here's where to go next.

1. **The 66-Day Rebuild System**
   The complete tracker for turning today's breakthrough into a locked-in habit — goal worksheets, daily logs, and the full 66-day structure. Inside the Stroked-Out Sasquatch Rebuild Tracker.

2. **The Recovery Activity Book**
   Cognitive and fine-motor practice — puzzles, aphasia-friendly word work, and hidden-object pages — for the days your rebuild needs a different kind of rep.

3. **The community**
   Other survivors running this same formula, comparing notes, and calling out excuses — find them through Stroke Recovery Academy.

4. **@strokedoutsasquatch on TikTok**
   Daily reality of the rebuild — drills, setbacks, and the Never Zero mindset in short form.

5. **strokerecoveryacademy.com**
   The full toolkit, the tracker downloads, and the rest of the story.

CLOSING

The Plateau Was Never the Ceiling

A plateau is not a verdict. It's a prompt — the system telling you it's time to Take a hard look, Learn something new, Experiment with courage, Combine what's already working, and Apply it like your identity depends on it. Because it does.

This formula is one piece of a much bigger toolkit. If it helped you break through, the full **Stroked-Out Sasquatch Rebuild Tracker** has the complete 66-Day Rebuild system, goal worksheets, a daily journal, and every other framework mentioned here — built to carry you past this plateau and the next one.

Come find the rest of the story, the community, and the tools at **strokerecoveryacademy.com**.



Never zero. One small meaningful action still counts.

STROKE RECOVERY ACADEMY

Your 66-Day Rebuild

A simple place to turn T.L.E.C.A. into a repeatable practice.

1 TAKE ? Name the exact thing that feels stuck.

2 LEARN ? Notice one signal from your body, task, or environment.

3 EXPERIMENT ? Choose one small, safe variable to change.

4 COMBINE ? Pair the rep with a cue, person, or routine.

5 APPLY ? Record what happened and choose the next rep.

66-DAY REBUILD TRACKER

Days 1·22 | Mark the rep, not perfection.

Day	One safe rep I chose	Done	Effort	Note
1				
2				
3				
4				
5				
6				
7				
8				
9				
10				
11				
12				
13				
14				
15				
16				
17				
18				
19				
20				
21				
22				

If today is a hard day: one honest rep still counts. If symptoms are new or worsening, stop and contact your care team.

66-DAY REBUILD TRACKER

Days 23·44 | Mark the rep, not perfection.

Day	One safe rep I chose	Done	Effort	Note
23				
24				
25				
26				
27				
28				
29				
30				
31				
32				
33				
34				
35				
36				
37				
38				
39				
40				
41				
42				
43				
44				

If today is a hard day: one honest rep still counts. If symptoms are new or worsening, stop and contact your care team.

66-DAY REBUILD TRACKER

Days 45·66 | Mark the rep, not perfection.

Day	One safe rep I chose	Done	Effort	Note
45				
46				
47				
48				
49				
50				
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66				

If today is a hard day: one honest rep still counts. If symptoms are new or worsening, stop and contact your care team.

STROKE RECOVERY ACADEMY

Weekly Review

Keep the evidence. Drop the judgment.

What became 1% easier this week?

What did I learn about timing, setup, fatigue, or support?

Which experiment is worth repeating next week?

What will I shrink on a hard day so the chain stays intact?